

# CMMS Consolidation — Business Case and Target Design

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Version 1.1, June 10, 2026. Prepared by Samuel Ibekwe, Director, Clinical Engineering, with Information Services. For decision by the executive team on June 24, 2026, and review by IS governance.

## 1. Summary and request

Clinical engineering equipment records are kept in two maintenance systems. This paper proposes moving all clinical engineering records into the enterprise CMMS already used by Facilities and by clinical engineering at the Fort Bend and outpatient sites.

We ask the executive team to approve the project and a one-time budget of \$186,000, covering vendor configuration and temporary data-entry support. Data cleanup would run July to October 2026 and migration of legacy records January to April 2027.

## 2. Current state

The legacy CE CMMS has held Medical Center West and Pavilion records since 2012. The enterprise CMMS has held Hospital Fort Bend, surgery center, Stafford and Mission Bend records since 2020, along with all facilities work orders.

At the end of May 2026 about 1,660 clinical devices were on file: roughly 960 in the legacy system and 700 in the enterprise system. One team of 13 staff works across both.

## 3. Problems with two systems

- Different work order steps: legacy PM work orders are created monthly and repair requests arrive by phone or email; the enterprise CMMS creates PM work orders ahead of due dates and takes portal requests.
- Recalls and alerts must be searched in both systems, and model names are recorded differently in each.
- The EDW loads a weekly enterprise CMMS extract, but the legacy system is exported by hand once a month, so PM completion for the two Harris County sites lags by up to a month.
- Risk categories and AEM category defaults were set up separately and do not match for about 40 device types.
- The legacy system runs on an on-premise server that reaches end of vendor support in 2027.

## 4. Target design

All clinical engineering devices, work orders and history would be held in the enterprise CMMS. Clinical engineering would use its own work order types for scheduled maintenance, corrective repair, incoming inspection and recall or alert actions.

- One device type list and risk classification across all sites, with AEM decisions recorded against device types.
- Manufacturer schedules kept for imaging and radiologic equipment, lasers and new devices without maintenance history, which cannot be placed on alternate maintenance.
- PM completion loaded to the EDW for all sites through the existing weekly extract, using the standard measure: PM work orders completed by due date over PM work orders due.
- Open design questions for the requirements review: asset tag format, whether facilities and clinical engineering share one instance, and how risk levels are recorded.

## 5. Data cleanup and migration

July to October 2026: reconcile device types, fill missing model and serial numbers, retire records for devices no longer in service and align risk categories. A temporary data-entry specialist supports the coordinator.

January to April 2027: load legacy devices and five years of work order history, run both systems in parallel for one PM cycle, then set the legacy system to read-only.

## 6. Costs and benefits

One-time cost is estimated at \$186,000. Retiring the legacy system avoids its annual maintenance fee and a server replacement. The main benefits are one process for technicians, timely reporting for all sites and a single search for recalls.

## 7. Risks

- Data quality in the legacy system is uneven; cleanup may take longer than planned.
- Technicians will carry both systems during parallel running.
- PM schedules must not slip during migration; high-risk PM stays at 100% on time throughout.